Patient Record — ID 2744

Age	42	**Sex**	F
Race/Ethnicity	African American	**Insurance**	Commercial PPO
Total Encounters	8	**Generation Seed**	3736

Primary Diagnoses

• Stress urinary incontinence
• Oligohydramnios
• Uterine leiomyoma
• Down syndrome

Comorbidities

• Personal History of Cholelithiasis
• Multiparity

Patient Profile

Sex	F
Age at First Encounter	42
Race/Ethnicity	African American
Insurance	Commercial PPO
Occupation	Marketing Consultant
Smoking Status	Never smoker
Alcohol Use	None

Allergies:

• NKDA

Chronic Conditions:

• Stress urinary incontinence
• Uterine leiomyoma
• Generalized anxiety disorder

Home Medications:

• Prenatal vitamin - 1 tablet daily

Surgical History:

• Repeat spontaneous vaginal deliveries (x4)
• Dilation and curettage for first-trimester miscarriage
• Second-stage labor complications (prolonged) with last delivery

Family History:

• Mother with history of uterine fibroids
• Negative for familial chromosomal abnormalities or genetic disorders

• No family history of Down syndrome

Encounters (8)

Encounter 1 — 2020-01-15 — Obstetrics & Gynecology

Encounter ID	11611	**Type**	outpatient
Attending	Dr. Sarah Chen	**Order**	0
Department	Obstetrics & Gynecology	**Gen. Method**	template

Chief Complaint:

Urinary leakage when exercising or sneezing that began after her last delivery two years ago.

Full Note:

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ENCOUNTER NOTE
Date: 2020-01-15
Provider: Dr. Sarah Chen, MD
Department: Obstetrics & Gynecology
Visit Type: outpatient

CHIEF COMPLAINT:
Urinary leakage when exercising or sneezing that began after her last delivery two years ago.

HISTORY OF PRESENT ILLNESS:
She reports that her symptoms began after the birth of her last child two years ago. She wears
protective pads daily and feels embarrassed during social outings due to unpredictable accidents.

PAST MEDICAL HISTORY:
Active Problem List:
- Stress urinary incontinence
- Uterine leiomyoma
- Generalized anxiety disorder

She has had four full-term vaginal deliveries.

PAST SURGICAL HISTORY:
- Repeat spontaneous vaginal deliveries (x4)
- Dilation and curettage for first-trimester miscarriage
- Second-stage labor complications (prolonged) with last delivery

MEDICATIONS:
Home Medications:
- Prenatal vitamin 1 tablet daily

ALLERGIES:
NKDA

FAMILY HISTORY:
- Mother with history of uterine fibroids
- Negative for familial chromosomal abnormalities or genetic disorders
- No family history of Down syndrome

SOCIAL HISTORY:
Smoking: Never smoker. Alcohol: None. Occupation: Marketing Consultant.

REVIEW OF SYSTEMS:
She denies nocturia, urgency, or dysuria.
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PHYSICAL EXAMINATION:
Pelvic examination shows mild uterine descent and slight anterior vaginal wall laxity. Post-void residual is 25 mL.

OTHER STUDIES:
Voiding diary reveals normal frequency with low-volume urine loss triggered by laughing or jumping.

ASSESSMENT AND PLAN:
Her symptoms are consistent with stress urinary incontinence

Begins to counsel her on behavioral strategies, including Kegel exercises, before considering medical or surgical interventions. The physician replies that pelvic floor exercises help reinforce the voluntary musculature that supports the urethra and bladder neck.

Encounter 2 — 2020-04-15 — Obstetrics & Gynecology

Encounter ID	11613	**Type**	outpatient
Attending	Dr. Sarah Chen	**Order**	1
Department	Obstetrics & Gynecology	**Gen. Method**	hybrid

Chief Complaint:

Six-month history of urinary frequency and urgency with progressive leakage when coughing or sneezing.

Full Note:

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ENCOUNTER NOTE
Date: 2020-04-15
Provider: Dr. Sarah Chen, MD
Department: Obstetrics & Gynecology
Visit Type: outpatient

CHIEF COMPLAINT:
Six-month history of urinary frequency and urgency with progressive leakage when coughing or
sneezing.

HISTORY OF PRESENT ILLNESS:
The patient returns for follow-up of stress incontinence evaluated in January 2020. She reports
that she experiences urinary leakage when coughing or sneezing, which has progressively worsened
over the past few months. She denies dysuria or hematuria.

PAST MEDICAL HISTORY:
Active Problem List:
- Stress urinary incontinence (diagnosed 2020-01-15)
- Uterine leiomyoma
- Generalized anxiety disorder

Her medical history is significant for menorrhagia and pelvic pressure for the past year. She has
not been pregnant before.

PAST SURGICAL HISTORY:
She denies any history of pelvic surgery.

MEDICATIONS:
Home Medications:
- Prenatal vitamin 1 tablet daily

ALLERGIES:
NKDA

FAMILY HISTORY:
- Mother with history of uterine fibroids
- Negative for familial chromosomal abnormalities or genetic disorders
- No family history of Down syndrome

SOCIAL HISTORY:
Smoking: Never smoker. Alcohol: None. Occupation: Marketing Consultant.

PHYSICAL EXAMINATION:
On physical examination, her abdomen is non-tender, and a bimanual pelvic exam reveals an
irregularly enlarged, non-tender uterus. There is no adnexal mass or tenderness.

LABORATORY DATA:
Her urinalysis is normal.
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Encounter 3 — 2020-07-15 — Urogynecology

Encounter ID	11610	**Type**	outpatient
Attending	Dr. Maria Rodriguez	**Order**	2
Department	Urogynecology	**Gen. Method**	hybrid

Chief Complaint:

Progressively worsening urinary leakage over the past six months when she coughs or exercises.

Full Note:

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ENCOUNTER NOTE
Date: 2020-07-15
Provider: Dr. Maria Rodriguez, MD
Department: Urogynecology
Visit Type: outpatient

CHIEF COMPLAINT:
Progressively worsening urinary leakage over the past six months when she coughs or exercises.

HISTORY OF PRESENT ILLNESS:
The patient returns for follow-up of urinary leakage seen in January and April. She reports that
this problem has been progressively worsening over the past six months.

PAST MEDICAL HISTORY:
Active Problem List:
- Stress urinary incontinence (diagnosed 2020-01-15)
- Uterine leiomyoma (diagnosed 2020-04-15)
- Generalized anxiety disorder

She has a history of two vaginal deliveries, the last of which was complicated by a prolonged
second stage of labor.

PAST SURGICAL HISTORY:
- Repeat spontaneous vaginal deliveries (x4)
- Dilation and curettage for first-trimester miscarriage
- Second-stage labor complications (prolonged) with last delivery

MEDICATIONS:
She is.

Home Medications:
- Prenatal vitamin 1 tablet daily

ALLERGIES:
NKDA

FAMILY HISTORY:
- Mother with history of uterine fibroids
- Negative for familial chromosomal abnormalities or genetic disorders
- No family history of Down syndrome

SOCIAL HISTORY:
Smoking: Never smoker. Alcohol: None. Occupation: Marketing Consultant.

REVIEW OF SYSTEMS:
Her menstrual periods are regular, and she denies any postmenopausal symptoms.

VITAL SIGNS:
Her BMI is 26 kg/m^2
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PHYSICAL EXAMINATION:
Abdominal examination is unremarkable. A cough stress test is positive for urine leakage.

LABORATORY DATA:
Urinalysis is normal.

Encounter 4 — 2023-01-15 — Obstetrics & Gynecology

Encounter ID	11617	**Type**	outpatient
Attending	Dr. Jennifer Park	**Order**	3
Department	Obstetrics & Gynecology	**Gen. Method**	hybrid

Chief Complaint:

Routine prenatal visit at 17 weeks' gestation with anxiety regarding chromosomal abnormalities given advanced maternal age.

Full Note:

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ENCOUNTER NOTE
Date: 2023-01-15
Provider: Dr. Jennifer Park, MD
Department: Obstetrics & Gynecology
Visit Type: outpatient

CHIEF COMPLAINT:
Routine prenatal visit at 17 weeks' gestation with anxiety regarding chromosomal abnormalities
given advanced maternal age.

HISTORY OF PRESENT ILLNESS:
The patient returns for routine prenatal care at 17 weeks' gestation with established histories of
stress urinary incontinence and uterine leiomyoma previously evaluated in this department in 2020.
She conceived spontaneously and has had an uncomplicated pregnancy thus far. She expresses anxiety
about the possibility of her child having a chromosomal disorder due to her age. She previously
declined first-trimester screening and is now interested in any tests that can still assess fetal
risk. She has no vaginal bleeding or abdominal pain and feels well overall.

PAST MEDICAL HISTORY:
Active Problem List:
- Stress urinary incontinence (diagnosed 2020-01-15)
- Uterine leiomyoma (diagnosed 2020-04-15)
- Generalized anxiety disorder

Her obstetric history includes one prior healthy vaginal delivery and one first-trimester
miscarriage.

PAST SURGICAL HISTORY:
- Repeat spontaneous vaginal deliveries (x4)
- Dilation and curettage for first-trimester miscarriage
- Second-stage labor complications (prolonged) with last delivery

MEDICATIONS:
She take any medications aside from prenatal vitamins

ALLERGIES:
NKDA

FAMILY HISTORY:
- Mother with history of uterine fibroids
- Negative for familial chromosomal abnormalities or genetic disorders
- No family history of Down syndrome

SOCIAL HISTORY:
She does not smoke, drink alcohol

PHYSICAL EXAMINATION:
Physical examination is unremarkable, and fetal heart tones are heard with Doppler. Fundal height
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is appropriate for gestational age.

PLAN:
The patient agrees to undergo second-trimester maternal serum screening, also known as the quad screen. When the test is performed, the clinician explains that the results can help identify the likelihood of neural tube defects and certain chromosomal abnormalities. If this fetus does in fact have trisomy 21,

Encounter 5 — 2023-01-15 — Obstetrics & Gynecology

Encounter ID	11615	**Type**	outpatient
Attending	Dr. Jennifer Park	**Order**	4
Department	Obstetrics & Gynecology	**Gen. Method**	hybrid

Chief Complaint:

Routine prenatal care at 17 weeks' gestation for ongoing pregnancy management.

Full Note:

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ENCOUNTER NOTE
Date: 2023-01-15
Provider: Dr. Jennifer Park, MD
Department: Obstetrics & Gynecology
Visit Type: outpatient

CHIEF COMPLAINT:
Routine prenatal care at 17 weeks' gestation for ongoing pregnancy management.

HISTORY OF PRESENT ILLNESS:
The patient returns for continued prenatal care at 17 weeks following recent assessment for
chromosomal concerns, with a history of stress urinary incontinence and uterine leiomyoma. She has
had an uncomplicated pregnancy so far.

PAST MEDICAL HISTORY:
Active Problem List:
- Stress urinary incontinence (diagnosed 2020-01-15)
- Uterine leiomyoma (diagnosed 2020-04-15)
- Down syndrome (diagnosed 2023-01-15)
- Generalized anxiety disorder

Her prior pregnancies were unremarkable with spontaneous vaginal deliveries at term.

PAST SURGICAL HISTORY:
- Repeat spontaneous vaginal deliveries (x4)
- Dilation and curettage for first-trimester miscarriage
- Second-stage labor complications (prolonged) with last delivery

MEDICATIONS:
Home Medications:
- Prenatal vitamin 1 tablet daily

ALLERGIES:
NKDA

FAMILY HISTORY:
- Mother with history of uterine fibroids
- Negative for familial chromosomal abnormalities or genetic disorders
- No family history of Down syndrome

SOCIAL HISTORY:
Smoking: Never smoker. Alcohol: None. Occupation: Marketing Consultant.

IMAGING:
A second-trimester ultrasound reveals increased nuchal translucency and a hypoplastic nasal bone.
The fetus also has shortened femur length and a possible endocardial cushion defect.

PLAN:
The patient is counseled about the possibility of chromosomal abnormalities and consents to a
maternal serum quad screen. Based on these findings,
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Encounter 6 — 2023-02-15 — Obstetrics & Gynecology

Encounter ID	11616	**Type**	outpatient
Attending	Dr. Amara Okafor	**Order**	5
Department	Obstetrics & Gynecology	**Gen. Method**	hybrid

Chief Complaint:

Follow-up prenatal visit at 17 weeks' gestation for routine surveillance and screening results.

Full Note:

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ENCOUNTER NOTE
Date: 2023-02-15
Provider: Dr. Amara Okafor, MD
Department: Obstetrics & Gynecology
Visit Type: outpatient

CHIEF COMPLAINT:
Follow-up prenatal visit at 17 weeks' gestation for routine surveillance and screening results.

HISTORY OF PRESENT ILLNESS:
The patient with a history of stress urinary incontinence, uterine leiomyoma, and generalized
anxiety disorder returns for prenatal follow-up at 17 weeks' gestation for routine surveillance
and screening results. She has had an uncomplicated pregnancy so far.

PAST MEDICAL HISTORY:
Active Problem List:
- Stress urinary incontinence (diagnosed 2020-01-15)
- Uterine leiomyoma (diagnosed 2020-04-15)
- Down syndrome (diagnosed 2023-01-15)
- Generalized anxiety disorder

Her prior pregnancies were unremarkable with spontaneous vaginal deliveries at term.

PAST SURGICAL HISTORY:
- Repeat spontaneous vaginal deliveries (x4)
- Dilation and curettage for first-trimester miscarriage
- Second-stage labor complications (prolonged) with last delivery

MEDICATIONS:
Home Medications:
- Prenatal vitamin 1 tablet daily

ALLERGIES:
NKDA

FAMILY HISTORY:
- Mother with history of uterine fibroids
- Negative for familial chromosomal abnormalities or genetic disorders
- No family history of Down syndrome

SOCIAL HISTORY:
Smoking: Never smoker. Alcohol: None. Occupation: Marketing Consultant.

IMAGING:
A second-trimester ultrasound reveals increased nuchal translucency and a hypoplastic nasal bone.
The fetus also has shortened femur length and a possible endocardial cushion defect.

PLAN:
The patient is counseled about the possibility of chromosomal abnormalities and consents to a
maternal serum quad screen. Based on these findings,
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Encounter 7 — 2023-05-15 — Obstetrics & Gynecology

Encounter ID	11612	**Type**	outpatient
Attending	Dr. Jennifer Park	**Order**	6
Department	Obstetrics & Gynecology	**Gen. Method**	hybrid

Chief Complaint:

Decreased fetal movements over the past 24 hours at routine third-trimester prenatal visit.

Full Note:

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ENCOUNTER NOTE
Date: 2023-05-15
Provider: Dr. Jennifer Park, MD
Department: Obstetrics & Gynecology
Visit Type: outpatient

CHIEF COMPLAINT:
Decreased fetal movements over the past 24 hours at routine third-trimester prenatal visit.

HISTORY OF PRESENT ILLNESS:
45-year-old with stress incontinence, leiomyoma, and anxiety returns for prenatal care. She
reports decreased fetal movements over the past 24 hours. Her pregnancy has been uncomplicated,
and she has received regular prenatal care.

PAST MEDICAL HISTORY:
Active Problem List:
- Stress urinary incontinence (diagnosed 2020-01-15)
- Uterine leiomyoma (diagnosed 2020-04-15)
- Down syndrome (diagnosed 2023-01-15)
- Generalized anxiety disorder

Her first pregnancy resulted in a spontaneous vaginal delivery at 39 weeks.

PAST SURGICAL HISTORY:
- Repeat spontaneous vaginal deliveries (x4)
- Dilation and curettage for first-trimester miscarriage
- Second-stage labor complications (prolonged) with last delivery

MEDICATIONS:
Home Medications:
- Prenatal vitamin 1 tablet daily

ALLERGIES:
NKDA

FAMILY HISTORY:
- Mother with history of uterine fibroids
- Negative for familial chromosomal abnormalities or genetic disorders
- No family history of Down syndrome

SOCIAL HISTORY:
Smoking: Never smoker. Alcohol: None. Occupation: Marketing Consultant.

PHYSICAL EXAMINATION:
On examination, her blood pressure is 120/78 mmHg, heart rate is 82 beats per minute, and fetal
heart rate is 140 beats per minute.

IMAGING:
An ultrasound is performed to assess amniotic fluid volume, which reveals an amniotic fluid index
(AFI) of 3 cm.
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OTHER STUDIES:
A nonstress test shows minimal fetal heart rate variability with no accelerations and no decelerations.

Encounter 8 — 2023-06-15 — Obstetrics & Gynecology

Encounter ID	11614	**Type**	inpatient
Attending	Dr. David Kim	**Order**	7
Department	Obstetrics & Gynecology	**Gen. Method**	hybrid

Chief Complaint:

Continued decreased fetal movements with confirmed oligohydramnios requiring admission for fetal monitoring.

Full Note:

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ENCOUNTER NOTE
Date: 2023-06-15
Provider: Dr. David Kim, MD
Department: Obstetrics & Gynecology
Visit Type: inpatient

CHIEF COMPLAINT:
Continued decreased fetal movements with confirmed oligohydramnios requiring admission for fetal
monitoring.

HISTORY OF PRESENT ILLNESS:
The patient with stress incontinence and leiomyoma returns for continued monitoring with an
uncomplicated pregnancy and regular prenatal care.

PAST MEDICAL HISTORY:
Active Problem List:
- Stress urinary incontinence (diagnosed 2020-01-15)
- Uterine leiomyoma (diagnosed 2020-04-15)
- Down syndrome (diagnosed 2023-01-15)
- Oligohydramnios (diagnosed 2023-05-15)
- Generalized anxiety disorder

PAST SURGICAL HISTORY:
- Repeat spontaneous vaginal deliveries (x4)
- Dilation and curettage for first-trimester miscarriage
- Second-stage labor complications (prolonged) with last delivery

MEDICATIONS:
Home Medications:
- Prenatal vitamin 1 tablet daily

ALLERGIES:
She has no known drug allergies.

FAMILY HISTORY:
- Mother with history of uterine fibroids
- Negative for familial chromosomal abnormalities or genetic disorders
- No family history of Down syndrome

SOCIAL HISTORY:
Smoking: Never smoker. Alcohol: None. Occupation: Marketing Consultant.

VITAL SIGNS:
On examination, her blood pressure is 122/78 mmHg, heart rate is 88 beats per minute, and fetal
heart rate is 140 beats per minute.

IMAGING:
An ultrasound is performed, which reveals an amniotic fluid index (AFI) of 4 cm.
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